2.2 Post-operative Care*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain near the wound is severe." • Rates it 8/10 **Objective** • Guards the incision • Grimaces on movement • HR 110/min, BP 145/90 mmHg	**ACTUAL** Acute pain related to surgical tissue trauma as evidenced by a pain score of 8/10, guarding and tachycardia.	• Pain at or below 3/10 within an hour of analgesia. • Tolerates deep breathing and early mobilization.	• Score pain on a numeric scale every 2–4 hours and after each intervention. • Give analgesics on time; with opioids, observe sedation level and respiratory rate. • Position for comfort and have him hold a pillow over the wound when coughing. • Time activity to coincide with peak analgesic effect. • Add simple non-drug measures: relaxation and a quiet bay.	• Considered met when the repeat score is 3/10 or less. • Uncontrolled pain prompts a review of the analgesic regimen with the physician.
Subjective • "I cannot cough because it hurts." **Objective** • Coarse crackles at both bases • Weak, unproductive cough • SpO_2 92% on room air; RR 24/min	**ACTUAL** Ineffective airway clearance related to residual anesthetic effects and pain-limited coughing as evidenced by crackles, an ineffective cough and reduced SpO_2.	• Clear breath sounds and SpO_2 of 95% or more within 24 hours.	• Auscultate and check SpO_2 every 4 hours. • Raise the head of the bed to 30–45 degrees. • Prompt deep breaths and a splinted cough every 1–2 hours. • Supervise incentive spirometry ten times an hour while awake. • Get him out of bed early, as ordered. • Give oxygen as prescribed.	• Achieved: chest clear and SpO_2 96%. • Retained secretions warrant chest physiotherapy and medical review.
Subjective • Feels light-headed on sitting up **Objective** • Four hours after abdominal surgery • Wound drain in place • On prophylactic anticoagulation	**RISK** Risk for excessive bleeding as evidenced by risk factors: recent surgery, surgical drain and anticoagulant therapy.	• Vital signs remain stable; dressing dry; drain output within the expected range.	• Record vital signs every 15 minutes for the first hour, then per protocol. • Inspect the dressing and drain; mark the level and chart the output. • Follow hemoglobin and coagulation results. • Escalate HR above 100/min, a falling BP or a rapidly filling drain. • Accompany his first walk, as postural hypotension is likely.	• Goal reached if observations stay stable and drainage remains small. • Active bleeding is reported to the surgeon at once.

* **References for this care plan:** [5], [14], [17], [35], [37], [93]. Full entries are listed in the References section of the CPMN book.